Durvalumab 10 mg/kg every 14 days

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Durvalumab consolidation · every 14 days

Active medicines: Durvalumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis for durvalumab alone; individual/local-policy rescue treatment if needed.
- No routine premedication specified; consider a reviewed plan after an infusion reaction.
- Steroids or immunosuppressants for immune toxicity require an individual consultant-led treatment and taper plan, not automatic prophylaxis.
- Provide immunotherapy alert card, emergency contacts, pulmonary-symptom advice and regular glucose/thyroid/organ-function testing.
- The preceding chemoradiotherapy has a separate supportive-care plan; do not carry its antiemetic steroids forward automatically.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00576 v5 · p7 · NCCP 00576 v5 supportive care

Sources checked 2026-10-29 · SACTCheck v0.84.26 · Regimen-level drafts; prescriber and oncology-pharmacy verification required